

ATTACHMENT 1
STATEMENT OF WORK (SOW)

SECTION I

GENERAL INFORMATION

1.1 INTRODUCTION

The National Institutes of Health (NIH) Clinical Center (CC) is America's clinical research hospital and the world's largest hospital dedicated totally to clinical research. Through clinical research, promising discoveries in the laboratory are translated into better health and health care for all. Currently, there are approximately 1500 active clinical research studies.

The NIH CC is a unique facility in that all patients are on a research protocol and many patients travel here from across the country and world and may be here for extended stays. Sometimes these patients are alone, may not have many visitors, and/or are without family members for support. Many patients with chronic or acute illnesses have depleted their financial resources and the cost of travel and lodging causes them additional stress. A positive experience with lodging accommodations is an important contributor to a patient's overall wellbeing. Because these patients play a critical role in the NIH mission, lodging costs may be subsidized by the NIH CC.

1.2 BACKGROUND

The vision of the NIH Clinical Center is to serve as the nation's premier research hospital for conducting clinical research to improve the health of humankind. It also serves as a national resource for clinical research by developing diagnostic and therapeutic interventions; enhancing systems to ensure the safe, efficient, and ethical conduct of clinical research; training clinical researchers; and leading the response to the nation's public health needs.

In 2005, the Mark O. Hatfield Clinical Research Center was opened. This state-of-the-art facility is home to inpatient units, day hospitals, outpatient clinics, and research labs. Hatfield Center connects to the existing Warren Grant Magnuson Clinical Center, which opened its doors to patients in 1953. Together, the Magnuson and Hatfield centers form the NIH Clinical Center, the world's largest clinical research complex. NIH serves dual roles, providing humane and healing patient care and an environment for clinical researchers to advance clinical science.

1.3 SCOPE

The scope of this project is to acquire hotel accommodations for NIH Clinical Center patients and/or family members/guardians at a fixed room rate, at or below the Government per diem lodging rate for zip code 20892 at www.gsa.gov.

Only medically stable patients who would normally be discharged to their home will be considered for lodging in a local hotel. The NIH CC cannot contractually commit to a fixed number of room nights per year due to individual patient preferences and variations in availability of funds for rooms subsidized by the NIH.

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1.4 LOCATION

These hotel facilities must be conveniently located, within a 7-mile radius to the National Institutes of Health (NIH) Clinical Center (CC), located at 10 Center Drive, Bethesda, Maryland 20892. Hotel locations are required to be within a 7-mile radius due to the need for patients/family members/guardians to travel to and from the NIH Clinical Center. This distance factors in accessibility to public transportation, distance, traffic and travel time.

1.5 OBJECTIVES

This project will achieve the following objectives:

1. Provide hotel rooms for NIH CC patients and/or family members/guardians when reservations are made by authorized NIH personnel.
2. Offer a fixed room rate that will be, at or below the government per diem rate for lodging at zip code 20892.
3. Hotel locations will be located within a 7-mile radius of the NIH Clinical Center's GPS location 5300 West Cedar Lane in Bethesda, MD 20892.

1.6 NIH CLINICAL CENTER KEY ROLES

Contracting Officer

The Contracting Officer (CO) will be responsible for resolving legal issues, determining contract scope, and interpreting contract terms and conditions. The CO is the sole authority authorized to approve changes in any of the requirements under this contract and, notwithstanding any clause contained elsewhere in this contract, the said authority remains solely with the CO. These changes include but will not be limited to the following areas: scope of work, price, quantity, technical specifications, and contract terms and conditions.

In the event the contractor effects any change at the direction of any person other than the CO, the change will be considered to have been made without authority and no adjustment will be made in the contract price to cover any increase in charges incurred as a result thereof.

The Contracting officer for this contract is:

NAME: Susan Nsangou

TITLE: Branch Chief

PHONE: 240-543-2479

EMAIL: susan.nsangou@nih.gov

OFFICE ADDRESS:

Office of Purchasing and Contracts

National Institutes of Health Clinical Center

6707 Democracy Boulevard, Suite 106, MSC 5480

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Contracting Officer's Representative

The Contracting Officer's Representative (COR) is responsible for the day-to-day technical guidance of the contract and is responsible for ensuring that the terms and conditions of this contract fully support the technical requirements specified herein. The COR shall be contacted regarding questions and/or problems of a technical nature. In no event will any understanding or agreement, modification, change order, or other matter deviating from the terms of the contract between the contractor and any other person other than the CO be effective or binding upon the Government.

When, in the opinion of the contractor, the COR requests effort outside the scope of the contract, the Contractor shall promptly notify the CO in writing. No action will be taken by the contractor under such technical instruction until the CO has determined if such effort is within the contract scope, and if not, has issued a contract change order.

On all problems that pertain to contract terms, the contractor shall contact the CO. The contractor shall contact the COR regarding problems or questions of technical nature. The COR is designated the Acceptance Official for all services rendered under this contract.

The following individual is designated as the COR:

NAME: Tony Richardson Deputy Chief, PSSD
ROLE: Contracting Officer's Representative (COR I)
PHONE: 301-496-3216
E-MAIL: tony.richardson@cc.nih.gov

OFFICE ADDRESS:

Patient Support Services Department
National Institutes of Health Clinical Center
10 Center Drive
Building 10, Room 1-5649
Bethesda, MD 20892

The following individual is designated as the Alternate COR:

NAME: Kara Smith
ROLE: Alternate Contracting Officer's Representative (COR I)
PHONE: 301-496-2341
E-MAIL: kara.smith@nih.gov

OFFICE ADDRESS:

Patient Support Services Department
National Institutes of Health Clinical Center
10 Center Drive
Building 10, Room 1-5649

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Contracting Officer's Representative Responsibilities

Specific COR responsibilities include, but are not necessarily limited to the following:

- a) Assuming responsibility for the inspection and acceptance of services in accordance with the contract terms and conditions addressing acceptance.
- b) Providing information to the contractor to assist in the interpretations of the specifications or technical portions of the Statement of Work.
- c) Monitoring services being performed to ensure that the contractor is meeting the requirements in the Statement of Work.
- d) Servicing as the contact point through which the contractor may relay questions or problems of a technical nature to the CO.
- e) Providing the CO with a copy of all correspondence which is or may be related to contractual matters.
- f) Submitting to the CO a written evaluation of the performance of the contractor. The written performance evaluation shall address effectiveness, quality, and timeliness.

Contracting Officer's Representative Limitations

Technical guidance shall be within the scope of work stated in the contract. The COR does not have the authority to issue any technical guidance that:

- a) Constitutes an assignment of additional work outside the scope of the contract;
- b) In any manner causes an increase or decrease in the total estimated contract performance; changes to any of the expressed terms, conditions, or specifications of the contract will be made by the CO.

SECTION II

TECHNICAL REQUIREMENTS

2.1 HOTEL AND ROOM REQUIREMENTS

- 2.1.1 The daily room rate shall be at or below Government General Services Administration (GSA) (www.gsa.gov) per diem lodging rate per night for zip code 20892. The NIH Clinical Center is a Federal Institution, and, therefore, is exempt from paying federal, state and sales taxes. Occupancy, local, or city fees may apply but no other fees or surcharges shall be authorized, such as credit card, phone, cable, room service, movies, and internet services. There shall be limited parking fees allowed with advance approval by a NIH official only at the time the reservation is requested.
- 2.1.2 Vendor provides guaranteed rates throughout the year, e.g. rates shall not fluctuate seasonally or when special events are being held.
- 2.1.3 The fully furnished hotel rooms shall include contractor provided utilities including heat, air conditioning, electricity, water and sewer, local telephone service (no deposit required) and basic television cable service.

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- 2.1.4 The hotel rooms shall comply with city, county and state building codes and ordinances applicable to the ownership and operation of the building in which the rooms are located.
- 2.1.5 The hotel shall be compliant with Hotel and Motel Fire Safety Act of 1990, 15 U.S.C. 2201 et seq, (Public Law 101-391), International Fire Code (IFC), and possess a FEMA Certification Number. The FEMA number can be verified at. [Hotel Help \(fema.gov\)](https://www.fema.gov)
- 2.1.6 The hotel shall be compliant with the American with Disability Act, 42 U.S.C. Section 12101 et seq.
- 2.1.7 Wheelchair accessible rooms shall be either on the ground floor or accessible an ADA compliant elevator. Access to rooms shall be from interior hallways that are accessed directly from the front desk/elevator lobby area. There shall be no physical barriers.
- 2.1.8 The hotel shall have handicap accessible features that include:
- Handicap accessible rooms
 - Handicap accessible bathrooms/showers
 - Handicap parking
 - Accommodations for guests who are deaf
- 2.1.9 The hotel may be required to have handicap accessible features that include:
- Accommodations for guests who are blind
- 2.1.10 The rooms shall be smoke-free unless requested by the patient/family member/guardian. Contractor personnel shall not smoke in the rooms at any time.
- 2.1.11 Refrigeration: Kitchenette and/or refrigerator for storing prescribed medications or special dietary products must be provided in the room, if requested.
- 2.1.12 Linens, towels, toiletries, and daily housekeeping service shall be provided for the rooms.
- 2.1.13 Iron and ironing board shall be provided in every room
- 2.1.14 A hair dryer shall be provided in every room.
- 2.1.15 Hotel(s) shall have 24/7 front desk coverage at the location where the patient/family members/guardian will be staying.
- 2.1.16 Parking shall be available for hotel guests who choose to drive.
- 2.1.17 Full-service restaurants shall be within close proximity to the hotel premises.
- 2.1.18 May provide shuttle services when available.

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2.2 ROOM RESERVATIONS

- 2.2.1 The reservation process must ensure that only authorized Government officials can make reservations as delineated in the Contract. NIH will not be held responsible for charges incurred by any individuals not indicated as authorized representatives of the NIH.
- 2.2.2 The full name of the NIH CC representative who authorized the reservation must be documented and included on the invoice submitted to NIH for payment.
- 2.2.3 The check in time for NIH CC guests shall be no earlier than 3 p.m. The checkout time for NIH patients and guests shall be no later than 12 p.m. Guests are permitted to check in earlier or check out later, subject to room availability no additional charge to NIH shall be assessed for these privileges.
- 2.2.4 Guaranteed reservations will be held until 10 p.m. on the scheduled arrival date. If the guest(s) does not arrive after 10 p.m. and is a no show, the Government will be charged a “no show charge fee” for that day and remaining days will be cancelled. The Government will not be charged by the Contractor for the remaining days of the reservation.
- 2.2.5 The contractor shall accept reservation cancellation(s) up to 6 p.m. on the day of the arrival. After 10 p.m., the Government will be charged a “no show charge fee” for that day and remaining days will be cancelled in accordance with paragraph 2.2.4.
- 2.2.6 There shall be no minimum number of days required per reservation.
- 2.2.7 In the event a patient/family member/guardian refuses to leave the room or leaves the hotel prior to the authorized days, the Contractor shall immediately notify the NIH Contracting Officer Representative.
- 2.2.8 Patient/family member/guardian may not have a credit card or money to provide as a deposit, at time of check-in, for amenities. All guests will be informed that they are responsible for paying for amenities. If someone checks out of the hotel without paying for some services, the NIH COR should be notified.

2.3 CUSTOMER SERVICE

- 2.3.1 The hotel employees are pleasant and efficient. They interact in a professional manner, listen to guest requests, and are responsive to requests for service.
- 2.3.2 Reservation services are accepted 24 hours a day.
- 2.3.3 All employees are efficient yet unhurried and sensitive to the manner of the guest.

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- 2.3.4 All employees provide a warm and sincere welcome greeting.
- 2.3.5 Guest is informed at time of check-in that NIH only pays for the room rate and any amenity charges will be billed to the guest. Guests with approved reservations under this contract will not be turned away if unable to provide a credit card or cash, at the time of check-in, for amenities.
- 2.3.6 Provide opportunity for guest to provide feedback regarding hotel stay.
- 2.3.7 The hotel must provide a customer service resolution plan and a quarterly report that supports customer satisfaction.

2.4 PATIENT EMERGENCIES

- 2.4.1 In the event a patient or family member has a medical emergency and is not able to call 911 on their own, the Contractor shall contact the 911 emergency medical services to respond to the emergency.

Once the emergency has ended, the Contractor shall notify the COR/ACOR during the business hours of 7:00 am to 4:30 pm. or Clinical Center Admissions after hours from 4:30 pm to 7:00 am at 301-496-3315.

2.5 ROOM MAINTENANCE

- 2.5.1 Daily room maintenance shall comply with all Federal, State and local laws and commercial standards. In the event of a guest reported maintenance issue, the appropriate repair person should respond and fix the issue within 60 minutes. If the issue cannot be resolved within 60 minutes the guest(s) shall be moved to a new room, if requested.
- 2.5.2 If maintenance needs to enter a guest room, the guest should be informed by the hotel front desk staff or manager the reason for entry and who will be coming to the room prior to maintenance personnel knocking on the door.

2.6 GENERAL BUILDING REQUIREMENTS AND SPECIFICATIONS

- 2.6.1 Heating and Cooling
 - 2.6.1.1 Heating and cooling requirements shall be in compliance with ADA and/or State or District of Columbia Laws.
 - 2.6.1.2 All cooling and heating devices and appliances, including but not limited to furnaces, fireplaces, electrical baseboard heaters and water heaters, shall be an approved type in good and safe working order and shall meet all installation and safety codes.
- 2.6.2 Electrical and Lighting

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- 2.6.2.1 Electrical and lighting requirements shall be compliant with ADA and/or State or District of Columbia Laws.
- 2.6.2.2 All electrical equipment, wiring and appliances shall be of an approved type, in good and safe working order and shall meet all installation and safety codes.
- 2.6.3 Sanitation requirements
 - 2.6.3.1 All plumbing fixtures shall be trapped and vented and connected to an approved sanitary sewer. All sanitary facilities, fixtures, equipment, structures and premises shall be of an approved type, in safe and sanitary condition and in good working order and meet all installation and safety codes.
- 2.6.4 Telephone Service requirements
 - 2.6.4.1 Contractor(s) shall provide local area, touch-tone, telephone service (no deposit required) for each room. The telephone service shall be capable of long-distance telephone service when long distance calls are placed using a telephone calling card or other telephone charge method.
 - 2.6.4.2 Contractor shall provide one telephone in each room
 - 2.6.4.3 Telephone service shall be equipped with voice messaging system or front desk message service.
- 2.6.5 Fire Safety
 - 2.6.5.1 Buildings in which space is offered shall be compliant in accordance with latest editions of the NFPA 101, IFC, and Life Safety Code and American with Disabilities Act (ADA)
 - 2.6.5.2 Equipment, services, or utilities furnished, and activities of other occupants shall be free of safety, health and fire hazards. When hazards are detected, the contractor shall promptly correct hazards.
 - 2.6.5.3 Maintenance of contractor-owned fire extinguishers shall be provided by the contractor in accordance with NFPA Standard No. 101.
 - 2.6.5.4 As required by code, smoke detectors/alarms shall be installed in each room, office, storage area, restaurants and kitchen, hallways and other common areas.

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2.6.6 Wheelchair accessibility

- 2.6.6.1 Common areas such as hallways shall provide for accessible path of travel for wheelchairs.

2.6.7 Elevators

- 2.6.7.1 If elevators are part of the room complex, they shall conform to the latest requirements of the American National Standard A171 and A171a Safety Code for Elevators, except for elevator emergency recall and shall be inspected and maintained in accordance with American National Standard A17.1, Inspector's Manual for Elevators or equivalent local code. The elevators shall also comply with local codes and ordinances.

2.6.8 Pest Prevention

- 2.6.8.1 The facilities are treated and maintained to ensure that they are free of vermin, bed bugs, cockroaches and other insects. Hotel must provide a pest prevention plan.

2.6.9 Construction/Remodeling

- 2.6.9.1 The vendor agrees to notify the COR/ACOR 30 days in advance or as needed, of any planned construction or remodeling to be performed in the facility which might be expected to generate or involve significant noise, dust, dirt, detours, or other disruption of use that may interfere with the housing of guests. The vendor agrees to notify the NIH CC immediately of any emergency construction to be performed in the facility which might be expected to generate or involve significant noise, dust, dirt, detours, or other disruption of use that may interfere with the housing of guests.

2.7 PERFORMANCE MONITORING

NIH Clinical Center will monitor contractor performance and certify the work is done in accordance with the contract. NIH COR will assure contractor performance through availability of lodging rooms, timeliness and accuracy of billing statements, inspections of contractor's site, and customer satisfaction reports made to NIH. The following criteria will be monitored to ensure satisfactory performance:

- a. Contractor shall provide a list of all hotel amenities. How many rooms in the hotel, how many are handicapped accessible, how many parking spaces, internet services, laundry, phones, elevators, wheelchair accessibility, etc. are available to guest and patients.

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- b. Provide proof that hotel facility and rooms comply with city, county, and state building codes and ordinances applicable to the ownership and operation of the building in which the rooms are located. Rooms shall comply with the applicable and current National Fire Protection Association (NFPA) Life Safety Code, Occupational Safety & Health Administration (OSHA) standards, and Americans with Disabilities Act (ADA) guidelines. Heating, electrical, sanitation and fire safety must all meet ADA and State of Maryland requirements. Contractor must provide proof.
- c. When the NIH CC Contracting Officer Representative (COR) receives a customer complaint, the Contractor must respond to the customer within 60 minutes and notify the COR by phone or email within 24 hours of notification and a written follow-up letter within five business days to the COR. All written correspondence including e-mail concerning guest under this contract shall be referred to as guest(s) and not patient(s).

SECTION III

3.1 DELIVERABLES

- 3.1.1 Hotel shall provide a monthly report by the 15th day of the month, for previous month's activity, to include:

Date	Day of Week	# of Nights (guest(s) arrived)	# of Cancelled Nights	# of No-Show Nights	Amount Billed

- a) date and day of week.
 - b) number of nights used.
 - c) # cancelled reservations.
 - d) # No-show reservations.
 - e) total billed amount; and
 - f) year-to-date totals for the contract year
- 3.1.1.2 The report should be created in Microsoft Office Excel and should be sent Via E-mail to the NIH CC Contract Officer Representative (COR). If a hotel has a different format that captures the same information, it can be discussed with the COR, at the time of award.
- 3.1.2.1.1.1 The Contractor shall respond to customer complaints within 60 minutes and notify the COR by phone or email within 24 hours of notification. A written follow up report/letter shall be submitted within five (5) business days to the COR. All written correspondence including e-mail concerning guest under this contract shall be referred to as guest(s) and not patient(s).
- 3.1.3 Hotel shall provide a Customer Service Resolution Plan and Quarterly Report.
- 3.1.4 Hotel shall provide a Pest Prevention plan.

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